

CLINICAL FINDINGS

Pemphigoid gestationis (PG) is exclusively associated with pregnancy. It typically presents during late pregnancy with the abrupt onset of intensely pruritic urticarial lesions. Fifty percent of patients experience first onset on the abdomen, often within or immediately adjacent to the umbilicus. The other half present with typical lesions but in an atypical distribution—such as on the extremities, palms, or soles. Rapid progression to a generalized, pemphigoid-like eruption is characteristic, usually sparing the face, mucous membranes, palms, and soles (although any site may be involved).

Flares occur with delivery in approximately 75% of patients and may be dramatic. An explosive onset of blistering can develop within hours of delivery, either as a flare of pre-existing disease or as a *de novo* presentation. Up to one-quarter of patients initially present during the immediate postpartum period.

Newborns may be affected in up to 10% of cases, but the disease is typically mild and self-limited. Immunofluorescence (IF) of newborn skin may yield positive findings despite a lack of clinically apparent disease, suggesting that more than C3 deposition alone is required for lesion formation.

Due to clinical and immunofluorescence similarities with bullous pemphigoid (BP), and because of longstanding confusion surrounding the term *herpes gestationis* (especially outside dermatology), most authors now use the term PG. However, several key differences exist:

1. BP primarily affects the elderly and shows no gender bias, whereas PG is exclusively associated with pregnancy.
2. PG shows a strong association with HLA-DR3, HLA-DR4, and a C4 null allele—associations not seen in BP.
3. Indirect IF is positive in most BP patients, with often high titers of anti-basement membrane zone (BMZ) antibodies. In PG, antibody titers are usually very low and require complement-enhanced techniques or ELISA for detection.

4. Most BP sera react with an intracellular 230- to 240-kD component of the hemidesmosome. In contrast, sera from most PG patients react with a 180-kD transmembrane protein containing a collagenous domain, encoded on a different chromosome.

Until disease classification is based more on pathomechanism than clinical appearance, diagnostic terminology will likely remain fluid.

Cutaneous Lesions

The typical lesions of PG are urticarial or arcuate plaques that rapidly evolve into a mixed dermatitis, including tense, pemphigoid-like blisters. Blisters may arise within urticarial plaques or on otherwise normal-appearing skin. Pruritic urticarial papules and plaques of pregnancy (PUPPP) can show microvesiculation but lack the tense, subepidermal blisters characteristic of PG.

Related Physical Findings

PG occurs exclusively in women and only in the context of pregnancy or the presence of trophoblastic tissue. Any other clinical setting is inconsistent with this diagnosis.

LABORATORY TESTS

Routine laboratory investigations are typically normal. Histopathology classically shows a subepidermal vesicle with a perivascular infiltrate of lymphocytes and eosinophils. Eosinophils may align along the dermal-epidermal junction and often fill the vesicular cavity. However, these classic findings are

seen in only a minority of cases. More commonly, a nonspecific mixed cellular infiltrate with variable numbers of eosinophils is observed.